

# Prioritization Decision Trees

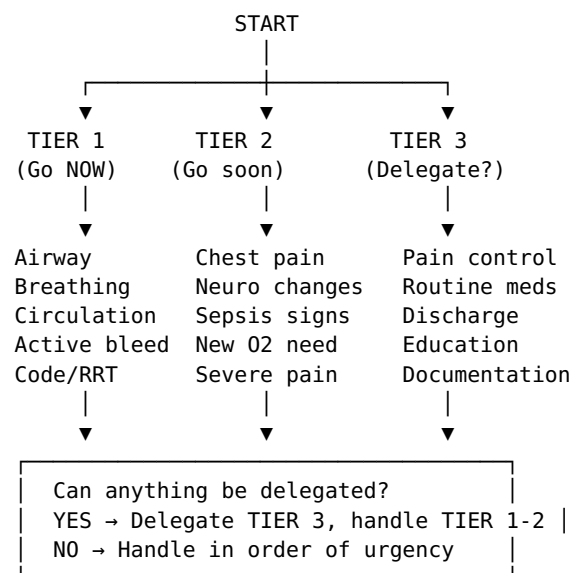
## Clinical Judgment Tools for NGN NCLEX and Practice

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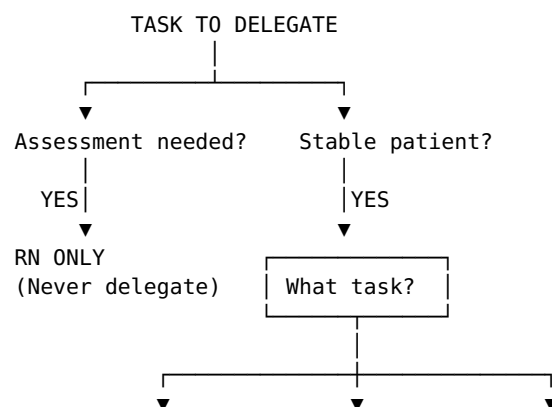
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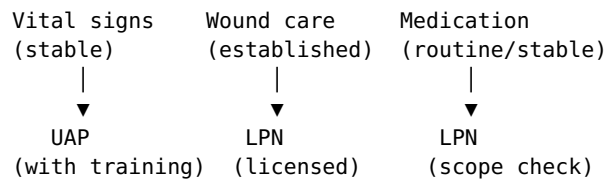
### Decision Tree 1: The “Who Do You See First?” Framework

When multiple patients need you and you’re the only nurse, use this tree:

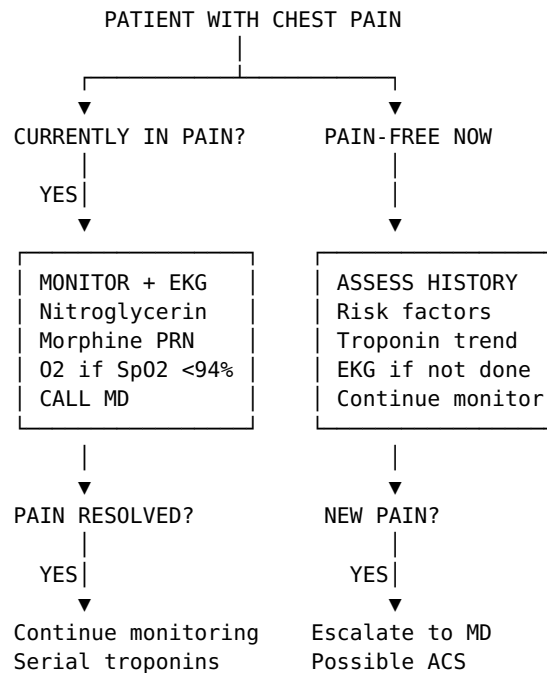


### Decision Tree 2: Delegation Flowchart

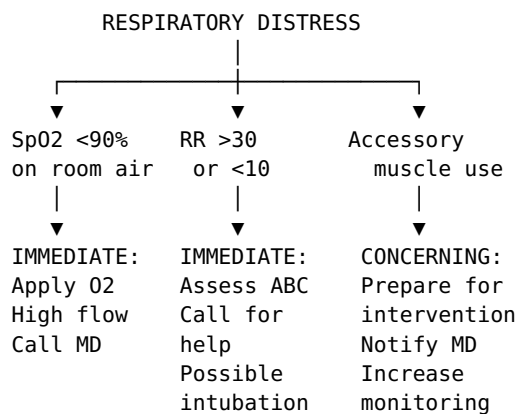




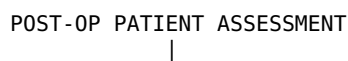
### Decision Tree 3: Chest Pain Triage

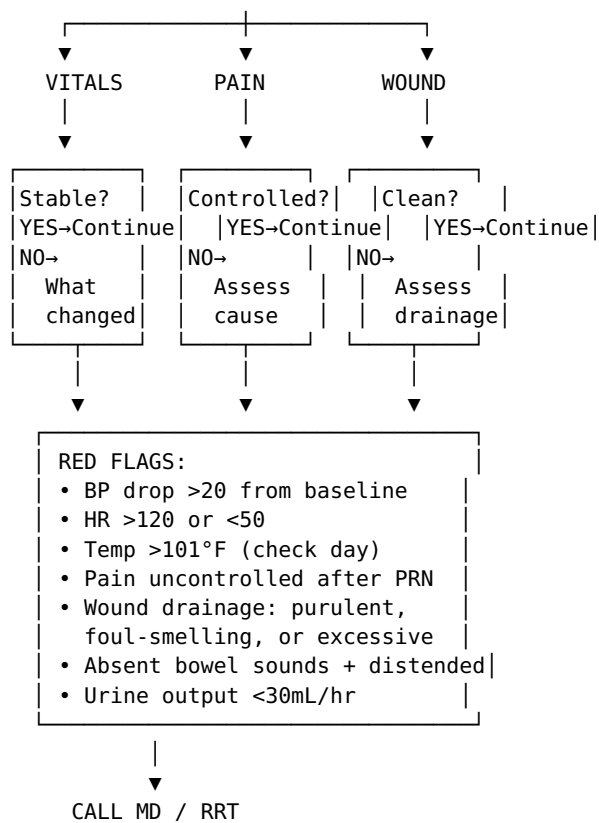


### Decision Tree 4: Respiratory Distress

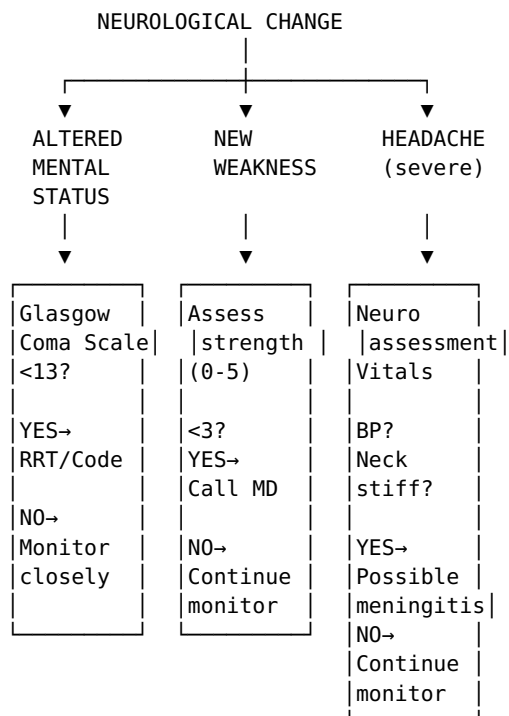


### Decision Tree 5: Post-Op Complications





## Decision Tree 6: Neurological Changes



## 10 Practice Scenarios

### Scenario 1: Medical-Surgical Floor

**Your assignment: 4 patients**

- **Room 1:** Post-op appendectomy, day 2. Pain 6/10, distended abdomen, no bowel sounds, last BM 3 days ago (pre-op).
- **Room 2:** CHF exacerbation, 82-year-old. SpO2 91% on 2L, RR 24, crackles bilaterally.
- **Room 3:** DKA, insulin drip running. Blood glucose 280, K+ 3.2, urine output 400mL last 8 hours.
- **Room 4:** Fall risk patient, mild dementia, trying to get out of bed, family at bedside.

**Question:** Who do you see first and why?

► Answer & Rationale

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### Scenario 2: Emergency Department

**You're triaging. Three patients arrive simultaneously:**

- **Patient A:** 34-year-old, "I think I have the flu." Temp 101.2, sore throat, otherwise healthy.
- **Patient B:** 67-year-old, "My chest feels heavy." BP 168/94, HR 96, pain 4/10, diaphoretic.
- **Patient C:** 5-year-old, fell from jungle gym. Crying, holding right arm, deformity visible.

**Question:** Triage order?

► Answer & Rationale

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### Scenario 3: Oncology Unit

**You have 3 patients:**

- **Patient A:** Neutropenic (ANC 400), temp 100.8°F. Last chemo 7 days ago.
- **Patient B:** Post-op Whipple procedure, day 3. NGT output suddenly increased, bilious, 800mL last 4 hours.
- **Patient C:** End-stage cancer, comfort measures only. Family requesting pain medication. Patient grimacing, moaning.

**Question:** Priorities?

► Answer & Rationale

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### Scenario 4: Code Blue Response

**You're a new grad. Code called on your unit. You arrive to find:**

- Patient unresponsive
- No pulse
- RN from next unit is running the code

- RT is managing airway
- Another nurse is doing compressions

**Question:** What do YOU do?

► Answer & Rationale

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[Continue with Scenarios 5-10 in complete product...]

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## Quick Reference Cards

### Vital Signs: When to Worry

| Vital Sign | Concern Threshold         | Action                         |
|------------|---------------------------|--------------------------------|
| SBP        | <90 or >180 (symptomatic) | Notify MD                      |
| DBP        | >110                      | Monitor, notify if symptomatic |
| HR         | <50 or >120               | Assess, notify MD              |
| RR         | <10 or >30                | Check SpO2, notify MD          |
| SpO2       | <92% on room air          | Apply O2, notify MD            |
| Temp       | >101°F (post-op day 1-3)  | Assess for infection           |
| Temp       | >100.4°F (neutropenic)    | <b>EMERGENCY</b>               |

### Lab Values: Critical Values

| Lab        | Low Critical | High Critical     | Common Cause                 |
|------------|--------------|-------------------|------------------------------|
| K+         | <2.5         | >6.5              | DKA, renal, diuretics        |
| Na+        | <120         | >160              | Dehydration, SIADH           |
| Glucose    | <50          | >400              | Insulin, DKA                 |
| Hgb        | <7           | >18 (dehydration) | Bleeding, anemia             |
| WBC        | <2,000       | >30,000           | Infection, immunosuppression |
| Platelets  | <20,000      | -                 | Bleeding risk                |
| Creatinine | -            | >4                | AKI, dehydration             |

*This is an excerpt. The complete system includes 30+ scenarios with full walkthroughs.*

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